



Data and Analytic Support for Healthcare (DASH) VISN 9 Pilot Kickoff

Presenters: Dr. Cory Fominaya and Blake Henderson

AGENDA

- Introductions
- Problem Statement
- What is DASH?
- Journey Map
- How does it work?(Function + Technical)
- Goals and KPIs for the Pilot
- Timeline
- Risks & Mitigations
- FAQs
- Next Steps
- Questions?

PROBLEM STATEMENT

Today we are asking clinical staff to make over 20M service-connection determination decisions a year for prescription benefits. On average clinicians make the right decision 75% of the time, leaving at least 5M questionable determinations.

WHAT IS DASH?

Data and Analytic Support for Healthcare (DASH) is a tool that uses a deep learning algorithm to identify and reduce prescription drug billing errors associated with a service-connected condition.

- Trained on 130M records since inception in 2017
- Completed 300K interventions

DASH was successfully piloted in VISN 9 in 2021 for a full year and paused to re-factor to a cloud architecture that would support the enterprise.

WHAT DOES IT DO?

- ❖ Improves the relationship of Veterans and the Centers that provide care
 - ✓ Increases accuracy of prescription billing
 - ✓ Reduces billing errors, Veteran confusion and calls to the VA
- ❖ Helps VA providers make more accurate benefit determinations
 - ✓ Introduces automation to support accurate benefit determinations
 - ✓ Decreases complaints/clinical interruptions/congressional inquiries

JOURNEY MAP

VA Clinician hurriedly makes a prescription benefit determination after seeing the Veteran, erroneously logging it as not service-connected and generating a co-pay.



Member Services fields a call from the frustrated Veteran. Member services can begin an audit process, which can take several weeks to reach a resolution.



A month later the Veteran receives a paper bill from VA for the co-pay that was charged and now they owe VA money.



The Veteran loses trust and confidence in the VA health system, and tells his friends he worries this will happen again in the future.

HOW DOES IT WORK? (FUNCTIONAL)

DASH detects disagreements between the provider and the ML model, called anomalies, and assigns a signal strength.



Some anomalies reach a **“high signal”** strength that will allow autonomous determinations.

Interventions occur in VistA autonomously behind the scenes

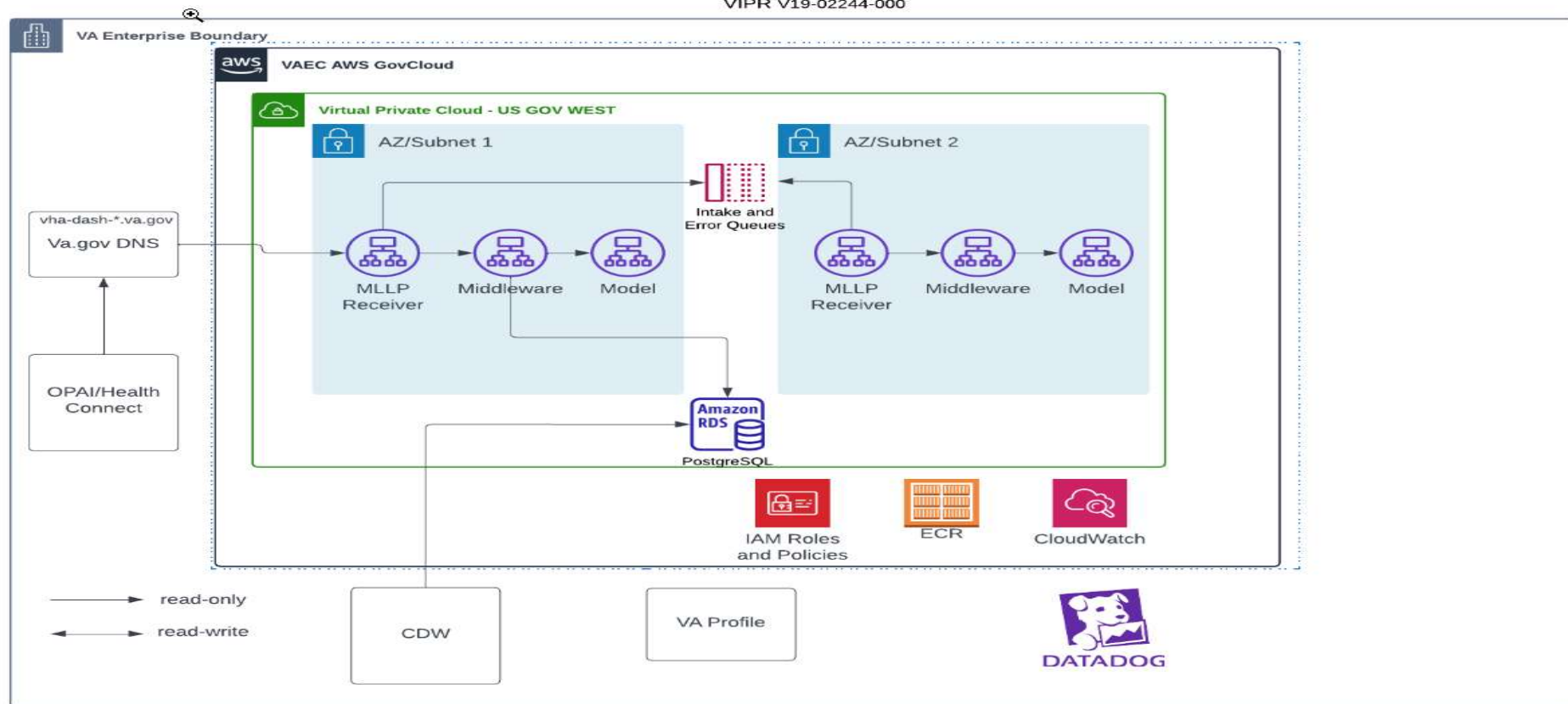
The prescription record is updated for the future and claims are submitted to insurance companies for medications that are not service-connected

No action is taken on the **Medium** and **Low** signal strength anomalies.

HOW DOES IT WORK? (TECHNICAL)

Data & Analytical Support for Healthcare (DASH)

VASI 2515
VIPR V19-02244-000



GOALS AND KPIs FOR PILOT

KPI	Definition	Estimated Output (VISN 9)
Detection of Anomalies	The discordance between the provider determination and the DASH determination	12,000 per week
Correction of Anomalies	Anomalies with “strong signals” that will be corrected autonomously by DASH	3,000 per week
Correction Accuracy Rate	The % of DASH corrections that are accurately made as determined by a statistically significant audit, estimated at n=400	Greater than 99%
Revenue Generated	The net change in VISN 9 revenue as a result of DASH’s autonomous interventions	\$9,600 per week

Risks & Mitigations

Risk	Mitigation	Owner
The DASH algorithm doesn't work correctly, possibly introducing additional errors in prescription billing.	- We will use human auditors from Revenue operations to evaluate DASH decisions in batches with a ceiling of n=400.	Cory Fominaya (Product Owner) & Georgette Diggs (Revenue Ops)
Increased Veteran Complaints due to unanticipated changes in prescription co-pays	- Monitor complaints monthly in Patient Advocate Tracking System - Monitor monthly Call Center call volume to the billing route using the VISN HRC Dashboard	Amanda Davis (VISN 9 VEO)
Clinician confusion due to a lack of visibility into DASH's actions	- The DASH team will make a dashboard available to VISN 9 stakeholders so they can view DASH interventions	Cory Fominaya

Communications

- How do we want to keep everyone updated?
 - Send an email to V9 stakeholders (same ones from June/July) so they are aware of go-live, include risk mitigation info
 - Meet bi-weekly during the 6-week pilot and then pivot from there.

DASH FAQs

- Will DASH correct service-connected and non-service connected prescriptions?
- How does DASH apply the corrections?
- Is DASH making the change in the system of record?
- What will the clinician see or experience when DASH makes a change?

NEXT STEPS

1. Obtain VISN 9 Pilot Approval (today)
2. Implement DASH tool (July 8th)
3. Schedule progress update meeting (early August)



Thank you.

<https://marketplace.va.gov/innovations/dash/>

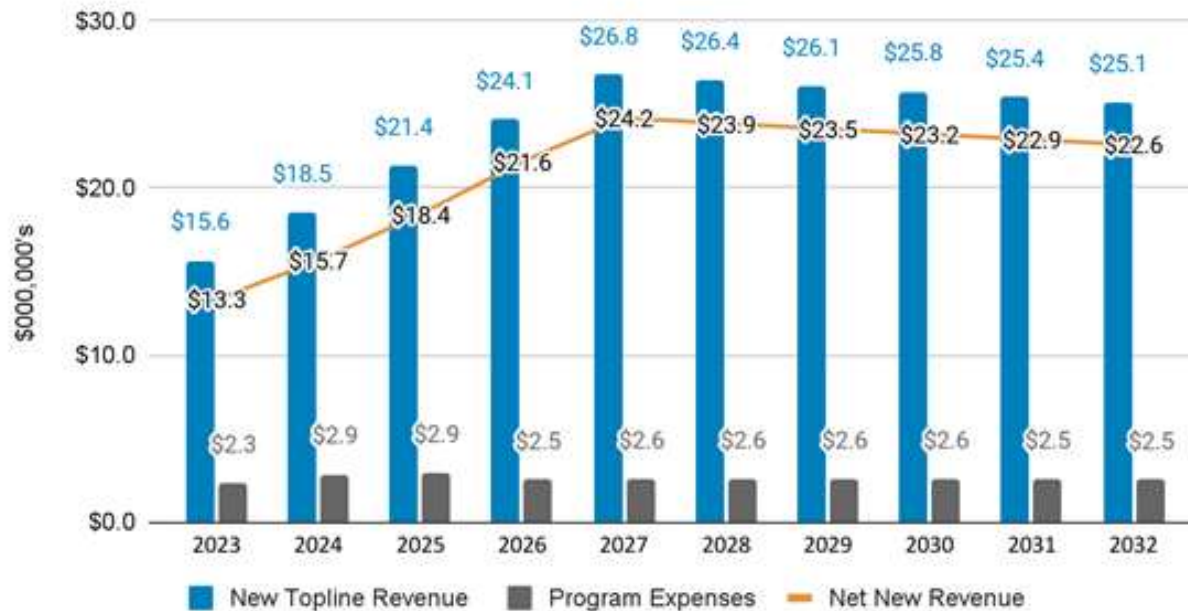
DASH Revenue Impact - VA Facility View



The average VA Facility would:

- Generate \$121,000 - \$207,000 per year in new topline revenue. Larger facilities would generate more than \$1M annually.
- Pay program expenses of \$18,000 - \$23,000 per year to the Franchise Fund for the DASH service.
- Generate \$103,000 - \$188,000 per year in net new revenue. Larger facilities would net up to \$1M annually.

Estimated DASH Net Revenue at Scale



After program expenses, DASH will generate \$13M - \$24M in net new revenue annually.

DASH Long-term Vision

DASH will be VA's **first end-to-end implementation** of an internally developed ML capability to improve administrative (i.e., non-clinical) operations.

We will sustain DASH as an **IT franchise fund product**, charging customers a percentage of the new revenue generated from co-pays.

DASH's business model and ATO can be **used repeatably** for follow-on non-clinical AI/ML models.